جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : فوزيه عبدالحافظ محمد -
المحافظة : الجيزه **الحي / المدينة :** الاخصاص **العنوان :** --------
رقم البطاقة : 26301010107588 **رقم التليفون :** 01118654775 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 01-01-1963 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة الاباتى المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فوزيه عبدالحافظ محمد -
National ID: 26301010107588

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bilateral mammogram and breast ultrasound

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
فوزيه
عبدالحافظ محمد السيد
٢٩ ش احمد حلمى-عزالدين عمر
الهرم - الجيزه
٢٦٣ ٠١ ٠١ ٠١ ٠٧٥ ٨٨
LU2288173


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رية منزل
انثى
مسلمة
متزوجة
السيد محمود احمد البوهى
البطاقة سارية حتى ٢٠٣٣/٠٥/٢٤
١

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Fawzeya Abdel Hafez Mohamed	Date : 20/02/2023
Exam : C.T CHEST WITH CONTRAST	Age : 8 Day(s)
Medical ID:	Pat. ID: 28951

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Left small enhanced LOQ breast focal lesion is noted, measuring about 12.5x13cm.
- Clear both lung fields. No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and right suprarenal.
- Bulky left suprarenal gland is seen.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Spondylodegenerative changes of the vertebrae.
- No lytic or sclerotic bone lesions.

CT Scan
[[DOCTYPE.CT_SCAN]]

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وزارة الصـــحة والسكـان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Fawzeya Abdel Hafez Mohamed	Date : 20/02/2023
Exam : C.T CHEST WITH CONTRAST	Age : 8 Day(s)
Medical ID:	Pat. ID: 28951

OPINION:

- Left breast small lesion for Sonomammaographic correlation.
- No pulmonary, hepatic or sclerotic deposits.

Dr. Alaa R. Hani, M.Sc., FEBR

Signature Prof. Menna Tallah Hanafy
25/02/2023 10:21 ص

CT Scan
[[DOCTYPE.CT_SCAN]]

Patient Name: Fawzia Abdel Hafez Mohammed **Date:** 12.06.2023

Mammographic Guided Stereotactic Wire localization:

Clinical summary: LT breast CA , For localization of left breast non palpable mass & 2 areas of coarse heterogeneous microcalcifications located at 12-1:00

Technique:

Under complete sterile conditions, 3ml of local anesthetic were administered at the site of concern in the LT breast. A 21G wire was used to localize the

Area 1: located at 1:00, 1.4cm deep to the skin, and 9cm from the nipple.

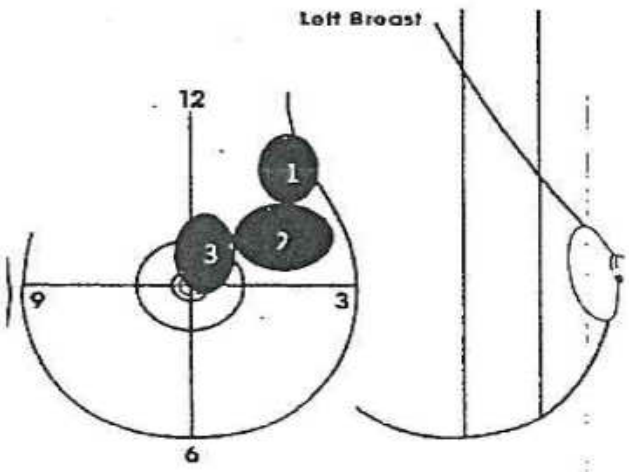
- The wire was inserted about 2cm from the skin mark over the area , the hook is in the centre of area of microcalcifications measures 2.3x1.9cm

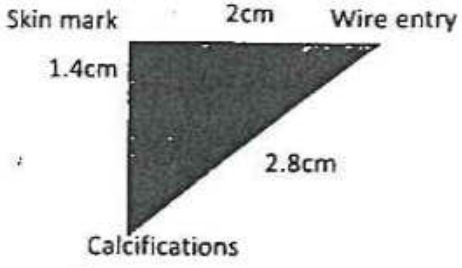
The mass : located at 1:00,1.7cm deep to the skin, and 6cm from the nipple.

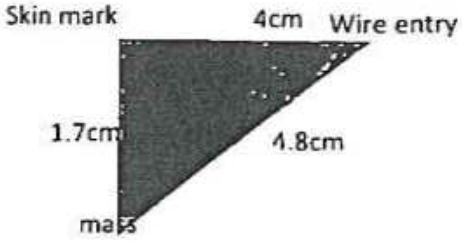
- The wire was inserted about 4cm from the skin mark over the clip, the hook is in the centre of the lesion

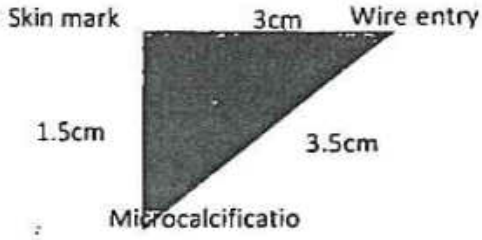
Area : located at 12:00, 1.5cm deep to the skin, and 4.5cm from the nipple.

-The wire was inserted 3cm from the skin mark, and the hook is in the centre of an area of microcalcifications measures2.1x1.3cm









Best Regards,
Dr Marwa Ramzy

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Women's Health Outreach Program

NAME: Foawzya Abdelhafez Mohamed

Date: Tuesday, July 02, 2019

High Resolution Ultrasonography

Clinical Presentation:

- Recalled from screening on 6/23/2019 for Two left UOQ clusters of amorphous calcifications
  - **Ultrasound Findings:**
  - Left area of altered hyperechogenecity with posterior acoustic shadowing at 1 o'clcok (corresponding to one of clusters of calcifications)
  - Left hetreogenous dense breast .
  - Left reactional intramammary LN at axillary tail.
  - Two cysts (0.5cm)seen at 12 o'clock with echogenic content in it, yet, no soft tissue component
  - Normal skin/nipple.
  - No pathological ax LN.

Conclusion:

- **Right Fibrocystic Breast with Two clusters of Mlaignant microcalcification for stereotactic guided wiring localization/ biopsy for Two clusters. B.I.R.A .D.S IV b**

Best regards,

DR: Nisreen Hassan

١٠٥٣ ش كورنيش النيل ـ ثم الخليج ـ القاهرة ـ جمهورية مصر العربية.
تليفون : ٢٣٦٠٥٨١٣
البريد الإلكتروني info@whop.gov.eg
الموقع الإلكتروني www.whop.gov.eg

Ultrasound
[[DOCTYPE ULTRASOUND]]

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Dar El Salam Cancer Hospital
Radiology Department

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أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Fawzeya Abd Al Hafez Mohamed Al Sayed	Date : 19/09/2024
Exam : C.T CHEST	Age : 61 Year(s)
Medical ID: 606/2023	Pat. ID: 30913

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

A known case of breast cancer with left BCS.

As compared to the previous study dated 23/08/2023, the current study shows:

1. Chest:

- Status post left BCS with evidence of reconstruction shows stationary course of the increased parenchymal thickness with decrease in size of the prominent left axillary level I lymph nodes.
- Clear both lung fields. No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal non enhanced CT appearance of the thoracic aorta and the great vessels.
- Upper abdominal cuts are remarkable
- Scanned skeleton shows: no sizable lytic or sclerotic bone lesions.

OPINION:

Status post left BCS shows:

- **No evidence of pulmonary metastatic disease or inflammatory process**

Dr: Ahmed Elgazar

Dr. mohamed Mohsen, M.D.

CT Scan
[[DOCTYPE.CT_SCAN]]

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Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Fawzeya Abd Al Hafez Mohamed Al Sayed	Date : 04/11/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 62 Year(s)
Medical ID: 606/2023	Pat. ID: 30913

BILATERAL BREAST HIGH RESOLUTION ULTRASOUND

Indication:

Patient had left CBS in

Technique

- Bilateral breast ultrasound

Report:

- eft breast UOQ 3 o'clock well defined heterogenous echogenic lesion measured 13x7mm likely fat necrosis
- left breast 4-5 o'clock focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific nght axillary LN, with preserved fatty hila and unitorm cortical thickness.
- Free left infra and supra clavicular regions.

Opinion:

- Left breast 3 o'clock well defined heterogenous echogenic lesion likely fat necrosis BIRADS 3)
- Left breast lower focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions
- Normal study of the right breast (BIRADS 1)

Yours truly

Signature Dr. basma mohmaed

04/11/2024 11:35 AM

Ultrasound
[[DOCTYPE ULTRASOUND]]

Patient Name: Fawzeya Abd Al Hafez Mohamed Al Sayed	Date : 30/09/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 62 Year(s)
Medical ID: 606/2023	Pat. ID: 30913

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Right BCS

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- Right diffuse skin thickening with underlying trabecular thickening with LIQ parenchymal distortion with increase in size and extent of the overlying micro-calcific foci probably of dystrophic nature. US showed 4:00 and Retroareolar area of parenchymal disruption owing to operative bed with no underlying solid or cystic lesions seen
- No suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed no solid or cystic mass lesions.
- Normal left skin thickness and nipple-areola complex.
- Left Nonspecific axillary LNs.
- Right no pathological lymph nodes

Opinion:

- Right postoperative changes with no underlying suspicious lesion or recurrence (BIRADS 2)
- Normal left breast study (BIRADS 1)

Dr. Mohamed Mohsen, M.D.

Immunohistochemistry
[[DOCTYPE IHC]]

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Radiology Department

خصصة
السلام
سعة

Patient Name: Fawzeya Abd Al Hafez Mohamed Al Sayed	Date : 30/09/2024
Exam : MAMMOGRAPHY BOTH	Age : 61 Year(s)
Medical ID: 606/2023	Pat. ID: 30913

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Left breast conserving therapy

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)

Left breast:

- Left breast para-areolar 12 o'clock heterogeneously echogenic lesion measuring 22 X 12 mm.
- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Right Breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post left BCS status showing operative bed para-areolar12 o'clock lesion (BIRADS 3) for close follow up.
- Normal study of the right breast (BIRADS 1)

Dr. Mohamed Mohsen, M.D.

Dr. Mohamed Mohsen, M.D.

Mammogram
[[DOCTYPE MAMMOGRAM]]

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

Exam Information:

Patient Name: Fawzia Abdullhafez	*Date of birth:* 01/01/1963
Arabic Name: فوزيه عبدالحافظ	*Patient Gender:* F
Patient RIS ID: 338105 *Patient HIS ID:* 123020270	*Exam Date:* 26/09/2023
Written by: Dr. Hoda Fathy	Approved by: Dr. Hoda Fathy

^{99m}Tc-MDP BONE SCAN

Brief history: 59 years old female patient diagnosed with managed left breast cancer.

Desired Information: - skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP in the following manner.

Report:

- *No hot spot that account for osseous deposits could be detected allover the surveyed skeleton.*
- *Spondylo-degenerative changes are seen over dorso-lumbar vertebrae.*
- *Pronounced osteoarthrosis noted over both knees joints.*
- *Both kidneys are normally visualized.*

Conclusion:

Negative study for the detection of any definite metastatic osseous deposits.

Thanks for reference

Nuclear Medicine Unit
Dina Thabet

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Kasr El-Ainy Center of Radiation Oncology and Nuclear Medicine

Faculty Of Medicine, Cairo University

"HEMROCK"

مركز قصر العيني

لعلاج الأورام والطب النووي

كلية الطب - جامعة القاهرة

(تمروك)

Nuclear Medicine Unit. Report

Patient Name:	Fawzya Abd Al-Hafez mohammad	Patient Gender:	Female
Patient Code:	Q36077	DATE:	3/22/2023 2:29:24 PM

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of left breast neoplasm.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- Focal tracer uptake is seen in the left frontal bone.
- Degenerative changes of the spine are noted, more pronounced in lumbar vertebrae.
- Osteoarthritic changes of both knees and shoulders are noted.
- The rest of the survey skeleton is unremarkable.

Opinion

- **Single worrying osseous lesion in the left frontal bone, likely metastatic for radiological correlation.** → traumatic

Dr. Nanys El Daly

Dr. Nahla Dessoki, MD

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Patient Name	: الأستاذه / فوزيه عبدالحافظ محمد السيد	Request No.	: 13002835
Patient No.	: 26301010107588	Request Date	: 13/06/2023
Path No.	: 1904/23	Verification Date	: 17/07/2023
Sex	: Female	Financial Category	: اورام دار السلام - تغطية الدولة
Age	: 59 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data: Left breast mass
Nature of the specimen:
Left wide local excision and sentenil lymph node biopsy specimens , collected at 13/6/23

Gross Examination:
Two specimens were recieved :
-Sentenil lymph node biopsy : Fibrofatty tissue peice measured 3x2 cm , enclosing 4 nodules up to 0.7 cm
-Left wide local excision specimen measured 10.5x8x3 cm with no covering skin elipse .Cut section revealed two wired firm greyish white masses. The first measured 2x1.5x1 cm , 0.5 cm away from least superficial margin.The second measured 1x1x0.5 , 1 cm away from least deep margin.
• Frozen section:
-Free SLNs(0/4)
-Free margins

Microscopic examination:
*First wired area:
Ductal carcinoma insitu (high grade)
*Second wired area
- Invasive duct carcinoma, Grade II with Predominant ductal carcinoma insitu (high grade).
• Angioinvasion: Not detected
• Lymph node involvement:Free (0/4)
• Resection Margins: Free all reescted margins
• Perineural invasion : Positive

Diagnosis:
Left breast mass , wide local excision and sentenil lymph node Biopsy specimens:

Verified by :
Dr.Marwa mahmoud

غرفة رقم (4) - الدور الأول - عمارة رقم (287) - شارع النيل كورنيش - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor Building No. 287- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: [illegible] / فوزيه عبدالفتاح محمد السيد	Request No	: 13007873
Patient No.	: 26301010107588	Request Date	: 29/01/2023
Path No.	: 263/2023	Verification Date	: 06/02/2023
Sex	: Female	Financial Category	: اورام ثدى السكر - تقة الدولة
Age	: 59 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast lesion

Nature of the specimen: Tru-cut biopsy, collected at 29/1/2022, 3:50 PM.

Gross Examination:
Seven grayish white and fatty tissue cores, 0.5 cm long each, totally submitted.
Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER PR HER2, ER clone (1D2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:
- Histological type: Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): II (3+2+1)
- Angioinvasion: Negative
- Ductal carcinoma in situ: Focal (comedo and cribriform)
- Additional pathologic findings: No microcalcification.
- IPT

ER: No nuclear reaction in tumor cells (Allred score 0/8)
PR: No nuclear reaction in tumor cells (Allred score 0/8)
Her-2: strong complete membrane reaction in >10% of tumor cells (score 3+)
Ki 67: 30 % nuclear reaction in tumor cells

Diagnosis:
Left breast lesion; Tru-cut biopsy:
- Invasive duct carcinoma, grade II
  ER: Negative (0/8)
  PR: Negative (0/8)
  HER2: Positive (score3+)

Verified by :
dr.esraa magdy

immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / فوزيه عبدالحافظ محمد السيد	Request No.	13007873
Patient No.	26301010107588	Request Date	29/01/2023
Path No.	263/2023	Verification Date	06/02/2023
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	59 Y	Department	Out Patient
Referreing Doc.			
		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

-Ki 67: 30 % nuclear reaction in tumor cells

Prof.Dr. Manal Elmahdy
Dr.Walaa Ghonaim

Dr.Israa Elgendy

Verified by :
dr.esraa magdy

غرفة رقم (4) - الدور الأول - مبنى رقم (357) - كورنيش النيل - ميت القبة - القاهرة - مصر
Room No. (4) - First Floor Building No. 357- Nile Corniche - Mit El Qubeta- Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

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وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Fawzeya Abd Al Hafez Mohamed Al Sayed	Date : 17/03/2025
Exam : ultrasound TRANS VAGINAL	Age : 62 Year(s)
Medical ID: 606/2023	Pat. ID: 30913

Trans-vaginal ultrasound scanning revealed: -

- *Uterus: -*
  - *AVF, average size uterus measuring about: 7x4x3cm in its longitudinal, transverse & AP diameters respectively.*
  - *The myometrium shows homogenous texture with no focal lesion*
  - *The endometrial line is 2mm.*

- *Ovaries: -*
  - *Both ovaries are average size with no focal lesion*

- ***No gross adnexal soft tissue masses or cysts.***
- ***No free fluid in the Douglas pouch.***

Conclusion:

Unremarkable study

Signature Dr . Nermien Ramadan
17/03/2025 12:37 PM

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Kasr El-Ainy Center of Radiation Oncology and Nuclear Medicine

Faculty Of Medicine, Cairo University

"NEMROCK"

Nuclear Medicine Unite

مركز قصر العيني

لعلاج الأورام والطب النووي

كلية الطب - جامعة القاهرة

(نمروك)

Report

Patient Name:	Fawzya Abd Al-Hafez mohammad	Patient Gender:	Female
Patient Code:	Q36077	DATE:	3/22/2023 2:29:24 PM

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of left breast neoplasm.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- Focal tracer uptake is seen in the left frontal bone.
- Degenerative changes of the spine are noted, more pronounced in lumbar vertebrae.
- Osteoarthritic changes of both knees and shoulders are noted.
- The rest of the survey skeleton is unremarkable.

Opinion

- Single worrying osseous lesion in the left frontal bone, likely metastatic for radiological correlation.

Dr. Nanys El Daly

Dr. Nahla Dessoki, MD

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
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وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Fawzeya Abdel Hafez Mohamed	Date : 20/02/2023
Exam : C.T CHEST WITH CONTRAST	Age : 8 Day(s)
Medical ID:	Pat. ID: 28951

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. *Chest:*

- Left small enhanced LOQ breast focal lesion is noted, measuring about 12.5x13cm.
- Clear both lung fields. No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. *Abdomen and pelvis:*

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and right suprarenal.
- Bulky left suprarenal gland is seen.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Spondylodegenerative changes of the vertebrae.
- No lytic or sclerotic bone lesions.

CT Scan
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ادارة الاشعة

Patient Name: Fawzeya Abd Al Hafez Mohamed Al Sayed	Date : 30/09/2024
Exam : MAMMOGRAPHY BOTH	Age : 61 Year(s)
Medical ID: 606/2023	Pat. ID: 30913

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Left breast conserving therapy

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)

Left breast:

- Left breast para-areolar 12 o'clock heterogeneously echogenic lesion measuring 22 X 12 mm.
- Operative bed parenchymal distortion and scarring with no sizable mass
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Right Breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post left BCS status showing operative bed para-areolar12 o'clock lesion (BIRADS 3) for close follow up.
- Normal study of the right breast (BIRADS 1)

Dr. Mohamed Mohsen, M.D.

Dr. Mohamed Mohsen, M.D.

Mammogram
[[DOCTYPE:MAMMOGRAM]]